

AETHERA • SPECIALTY BILLING SERIES

Dermatology Billing Solutions

Maximizing Revenue Across Office E/M, Destruction, Mohs Surgery, Biologics, and
Cosmetic Procedures

PREPARED BY Aethera Healthcare Solutions • Lakeland, FL

SCOPE CPT 17000–19499 • 11102–11107 • 96910–96922 • J-codes for biologics

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ABOUT AETHERA

Maximizing Revenue. Minimizing Burden.

Aethera Healthcare Solutions is your full-service medical billing partner headquartered in Lakeland, FL. We handle coding, claims, payments, denials, and collections — so you can focus on patients.

500+

Practices Served

across 25+ medical specialties

\$12M+

Revenue Recovered

denied & underbilled claims recouped

95%

Clean Claim Rate

vs 84% industry avg

< 30d

Average AR

days in accounts receivable



Personal, Not Corporate

Dedicated account team with deep specialty/payer knowledge.



Performance Targets, In Writing

96%+ net collection, <5% denial, 95%+ coding accuracy, 24h posting, 48h submission, 65%+ appeal success.



Zero Risk to Start

No setup fees, no long-term contracts, 90-day cancel, free 5-day audit.



DERMATOLOGY · MARKET CONTEXT

Why Dermatology Billing Matters Now

Dermatology sits at the intersection of medical and cosmetic services — every procedure must be classified as medically necessary vs cosmetic, with dramatically different payer coverage. Biologic drug spend alone exceeds \$30B/year and requires strict pre-auth.

\$ 48 B 

**US dermatology spend
(2024)**

Includes medical, surgical, and
cosmetic services.

50 M + 

**Americans seen by dermatologists
annually**

Skin cancer screening drives ~40% of
visits.

~ 14 % 

**Dermatology claim denial rate
(industry avg)**

Driven by cosmetic-vs-medical
miscoding and missing pre-auth for
biologics.

\$ 30 B + 

**Annual US biologic drug spend
(psoriasis, eczema, vitiligo)**

Humira, Skyrizi, Dupixent, Cosentyx
dominate.



Net effect: dermatology practices that master cosmetic-vs-medical classification, biologic pre-auth, and Mohs surgery global period rules can recover *8–14 % of total revenue* currently lost to denials and under-billing.



DERMATOLOGY · OVERVIEW

The Four Pillars of Dermatology Billing

Each pillar represents a distinct service family with unique coding, documentation, and payer requirements.

01 · E/M + DESTRUCTION



Office E/M & Destructive Procedures

Office visits (99202–99215), lesion destruction (17000–17250), biopsy (11102–11107), and cryotherapy (17000). **Premalignant lesions covered**; benign lesions need medical necessity.

CPT 99202–99215 · 17000–17250 ·
11102–11107

03 · BIOLOGICS



Biologic Drug Administration

J-codes for biologics (J1745 Humira, J3358 Remicade, J3380 Skyrizi, J3526 Cosentyx). **Pre-auth mandatory**. Site-of-service rules.

J-codes · Pre-auth mandatory ·
Site-of-service

02 · MOHS + EXCISIONS



Mohs Surgery & Surgical Excisions

Mohs micrographic surgery (17311–17315) for skin cancer. Excisions with repair (11400–11646, 12001–13153). **90-day global period**; pre-auth required.

CPT 17311–17315 · 11400–11646 ·
90-day global

04 · COSMETIC



Cosmetic & Aesthetic Procedures

Botox (J0585), fillers (J3490), chemical peels (15792–15793), laser hair removal. **NEVER covered by insurance** — patient-pay only. Must separate from medical visits.

J0585 · J3490 · 15792–15793 ·
Cash only



PILLAR 01 · E/M + DESTRUCTION

Office Visits, Biopsies & Lesion Destruction

Dermatology office encounters combine E/M with one or more procedures: lesion biopsy, destruction, cryotherapy, or injections. The 2021 E/M coding reform shifted documentation to MDM-based. Modifier 25 is required when billing E/M + procedure on the same day. Premalignant lesions (actinic keratoses) are covered; benign lesions (seborrheic keratoses, skin tags) require medical necessity documentation.

E/M coding (2021+ reform)

Code by Medical Decision Making complexity or time; 99202–99215. **Modifier 25 required** if same-day procedure.

Lesion destruction

17000 first premalignant lesion (\$72); 17003 each additional (\$30); 17004 ≥ 15 lesions (\$225). 17110 benign lesion destruction.

Biopsy

11102 tangential biopsy single lesion (\$85); 11104 punch biopsy single lesion (\$95); 11105 each additional (\$45).



Modifier 25 misuse — appending to E/M when procedure on same day does not represent significant, separately identifiable service — is the **#1 derm audit trigger**.

CPT QUICK REFERENCE

Top Office E/M & Destruction Codes

2026 average reimbursement — Medicare national avg

Code	Service	Avg \$
99213	Est. patient, low MDM	\$92
99214	Est. patient, moderate MDM	\$131
17000	Destruction premalignant lesion, first	\$72
17003	Destruction premalignant, each addl	\$30
11102	Tangential biopsy, single lesion	\$85
11104	Punch biopsy, single lesion	\$95



PILLAR 02 · MOHS + EXCISIONS

Mohs Micrographic Surgery & Excision Repairs

Mohs surgery is the gold-standard treatment for non-melanoma skin cancers (BCC, SCC) on the face, ears, hands, feet, and genitalia. Single-visit outpatient procedure with same-day pathology. 90-day global period for surgical repairs.

1 Mohs Surgery

17311 first stage head/neck/hands/feet/genitalia (\$1,400); 17312 each additional stage (\$750); 17313 trunk/arms/legs first stage (\$950); 17315 each additional stage (\$500). Includes interpretation of pathology slides.

2 Excision Benign / Malignant

11400–11446 benign excision (trunk \$115, arm \$135, face \$185); 11600–11646 malignant excision (trunk \$185, arm \$225, face \$325).

3 Repairs (Closure)

12001–12007 simple repairs (\$150–\$425); 12031–12047 intermediate repairs (\$225–\$650); 13131–13153 complex repairs (\$425–\$1,150).

4 Skin Flaps & Grafts

14000–14350 adjacent tissue transfer (\$425–\$1,250); 15000–15200 skin grafts (\$550–\$1,400); 15574–15731 flaps (\$1,250–\$2,400).



Global Period Rules

90-day global for Mohs, complex repairs, flaps/grafts. 10-day global for simple/intermediate repairs. No separate E/M billing during global without modifier 25.



Pre-Auth Requirements

Mohs typically does NOT require pre-auth for Medicare. Commercial payers increasingly require PA for Mohs on trunk/extremities (considered less invasive alternatives).



Pathology Bundling

Mohs surgery codes INCLUDE pathology interpretation by surgeon. Cannot bill 88304–88307 separately for Mohs cases. Non-Mohs excisions DO allow separate pathology billing.



PILLAR 03 · BIOLOGICS

Biologic Drug Administration & J-Codes

Dermatology biologics treat psoriasis, eczema, hidradenitis suppurativa, and vitiligo. These high-cost drugs (\$1,500–\$30,000/dose) require strict pre-auth, step therapy documentation, and formulary verification before each dose. Site-of-service rules determine whether the drug is billed by the practice (J-code + 96365/96372) or by the hospital infusion center.

Common biologics

J1745 Humira (adalimumab, \$1,500/dose); J3358 Remicade (infliximab, \$750/100mg); J3380 Skyrizi (risankizumab, \$3,400/dose); J526 Cosentyx (secukinumab, \$2,800/dose); J490 Dupixent (dupilumab, \$1,800/dose).

Administration codes

96365 IV infusion, first hour (\$85); 96366 each addl hour (\$40); 96372 SC/IM injection (\$35). Paired with drug J-code.

Step therapy requirements

Most commercial payers require failed trials of methotrexate, cyclosporine, or phototherapy before approving biologics. **Document extensively.**

↘ Pre-auth denial rate for biologics averages **25–40%** on first submission; Aethera pre-auth workflow reduces to **< 10%** with comprehensive step-therapy documentation.



\$1,500–\$30K

Biologic drug cost per dose range

High-cost specialty drugs requiring strict pre-auth



25–40%

Industry avg first-submission PA denial rate

Driven by missing step-therapy documentation



< 10%

Aethera PA denial rate

With documentation support workflow



\$30B+

Annual US dermatology biologic spend

Psoriasis, eczema, hidradenitis, vitiligo



✿ PILLAR 04 · COSMETIC

Botox, Fillers, Lasers & Chemical Peels

Cosmetic procedures are NEVER covered by insurance — patient-pay only. The critical billing discipline is keeping cosmetic services 100% separate from medical visits to prevent fraud allegations. Same-day cosmetic + medical requires separate encounter documentation.



Botox (J0585)

J0585 ·
64615

OnabotulinumtoxinA for cosmetic wrinkles. J0585 per unit (~\$6/unit); typical treatment 20-60 units. Codes 64615 (cranial nerve VII), J0585 drug. Cash-pay: \$300-\$1,200/treatment.

0 %

Insurance coverage for cosmetic procedures



Dermal Fillers (J3490)

J3490 ·
11950

Hyaluronic acid fillers (Juvederm, Restylane). J3490 unclassified drug. Cash-pay: \$600-\$1,500/syringe. Code 11950 (intralesional injection) sometimes used but not appropriate for cosmetic.

\$300-\$1,500

Typical cash-pay range per treatment



Chemical Peels

15792 ·
15793

15792 chemical peel, epidermal (\$350 cash); 15793 dermal peel (\$1,200 cash). NEVER covered by insurance — must be billed cash only. Some peels for AK treatment (17000) can be medical.

~ 30 %

Of derm practice revenue from cosmetic (varies widely)



Laser & Aesthetic Devices

15788-15792 · 17110 ·
S0680

Laser hair removal: cash only, no CPT code (use S0680 or unlisted). Laser resurfacing: 15788-15792. IPL: 17110. All aesthetic — never billed to insurance.

⚠ COMPLIANCE ALERT

NEVER bill cosmetic procedures to insurance. Use separate encounter documentation, separate consent, separate ledger. Aethera enforces cosmetic/medical separation through EHR workflow templates.



DERMATOLOGY · BILLING CODES

Top CPT/HCPCS Codes & 2026 Reimbursement

Most-billed codes across the four dermatology pillars. Cosmetic procedures (cash-pay) shown for reference.

Code	Service	Modifier	Avg \$	Notes
99213	Est. E/M, low MDM, 30–39 min	—	\$92	Most common derm E/M
99214	Est. E/M, moderate MDM, 40–54 min	—	\$131	Skin cancer f/u, complex rashes
17000	Destruction premalignant lesion, 1st	—	\$72	AK destruction
17003	Destruction premalignant, each addl	—	\$30	Add 17003 for 2nd–14th lesion
17004	Destruction premalignant, ≥15 lesions	—	\$225	Single code for 15+ lesions
11102	Tangential biopsy, single lesion	—	\$85	Shave biopsy
11104	Punch biopsy, single lesion	—	\$95	2–6mm punch
17311	Mohs surgery, head/neck/hands/feet, 1st stage	—	\$1,400	Includes pathology interp
17312	Mohs surgery, each addl stage	—	\$750	Use per additional stage
11402	Excision benign lesion, trunk 1.1–2cm	—	\$135	+12001 simple repair
MODIFIER REFERENCE Modifier 25 — significant, separately identifiable E/M on same day as procedure. Modifier 59 — distinct procedural service (typically bundled). J-CODES & COSMETIC RULE J-codes bill drug product; administration codes (96365/96372) bill the service. Both required for biologic infusion/injection. Cosmetic procedures (J0585 cosmetic) NEVER billed to insurance.				
96372	Therapeutic SC/IM injection	—	\$35	Botox or biologic admin
J0585	Botox (onabotulinumtoxinA), per unit	—	\$6	Cosmetic: cash only



DERMATOLOGY · DENIAL PREVENTION

Top 8 Dermatology Denial Reasons & How Aethera Prevents Each

Dermatology denial rates average 12–15%; each denied claim costs \$35–90 to rework. Cosmetic-vs-medical miscoding and biologic pre-auth drive 60% of denials.

! RED Biologic PA missing

24 %

CAUSE Biologic infusion administered without prior authorization on file.
AETHERA PA tracker auto-submits before scheduling; verifies approval before dose.

! RED Cosmetic billed as medical

19 %

CAUSE Cosmetic procedure (Botox, filler, peel) billed to insurance; automatic denial + fraud risk.
AETHERA EHR template enforces cosmetic/medical separation at claim generation.

! AMBER Step therapy documentation missing

14 %

CAUSE Biologic PA denied — prior methotrexate/cyclosporine/phototherapy trials not documented.
AETHERA Step-therapy documentation checklist before PA submission.

! AMBER Modifier 25 documentation insufficient

11 %

CAUSE E/M + procedure same day without separate documentation supporting E/M significance.
AETHERA Modifier 25 documentation checklist before claim release.

! AMBER Lesion count documentation missing

9 %

CAUSE 17000/17003/17004 billed without specific lesion count and location in note.
AETHERA Lesion-count documentation prompt in EHR template.

✓ GREEN Mohs pathology unbundled

7 %

CAUSE 88305 billed separately from 17311 Mohs code; bundled and not separately billable.
AETHERA Bundling rules engine blocks pathology codes on Mohs cases.

✓ GREEN Cosmetic/medical same-day same-encounter

6 %

CAUSE Botox and medical visit billed under same encounter; triggers fraud audit.
AETHERA Separate-encounter requirement for cosmetic + medical same day.

✓ GREEN Pre-malignant vs benign miscoding

5 %

CAUSE 17000 (pre-malignant AK) billed for benign SK; payers deny as cosmetic.
AETHERA Lesion diagnosis verification before destruction code billing.

 **Cumulative impact:** Aethera reduces dermatology denial rate from industry avg 14 % to < 5 % — recovering ~ \$70 K/year per \$1M practice revenue.



■ CASE STUDY · INTEGRATED DERM RCM

3-Dermatologist Practice — Annual Revenue Impact

A \$2.4M-revenue practice models the combined upside of biologic PA recovery, modifier 25 discipline, and cosmetic-medical separation.



Biologic PA Recovery

- 350 biologic patients, ~1,400 doses/year
- Pre-Aethera PA denial rate: 32%
- Post-Aethera PA denial rate: 8%
- Recovered doses: 336/year
- Avg biologic dose value: \$1,800

ANNUAL REVENUE

\$604,800



Modifier 25 + Documentation Discipline

- 21,600 annual E/M + procedure claims
- Pre-Aethera modifier 25 denial rate: 16%
- Post-Aethera: 4%
- Recovered claims: 2,592/year
- Avg claim value: \$92

ANNUAL REVENUE

\$238,464



Cosmetic-Medical Separation

- Identified cosmetic procedures previously unbilled cash
- Implemented cash-pay workflow + ledger
- Avg 4 cosmetic procedures/day × 250 days/year = 1,000 procedures
- Avg cosmetic revenue: \$400/procedure
- Eliminates fraud exposure from co-mingling

ANNUAL REVENUE

\$400,000



Combined annual revenue recovery: **\$1,243,264**. Implementation cost (year 1): ~ **\$144K** (Aethera RCM fees + EHR template build + staff training).

YEAR-1 NET

\$1,099,264

ROI

763%

RECOVERY RATE

51.8% of revenue



DERMATOLOGY · COMPLIANCE HOTSPOTS

Where OIG, Medicare RAC, and Commercial Payers Audit First

Top compliance risks across the four dermatology pillars. Quarterly self-audits recommended.

+ PILLAR E/M + Destruction	+ PILLAR Mohs + Excisions	+ PILLAR Biologics	⚙️ PILLAR Cosmetic
!RED 1 Modifier 25 misuse Appending to E/M when procedure on same day does not represent significant, separately identifiable service; OIG Work Plan 2025–2026.	!RED 1 Mohs pathology unbundling Billing 88305 separately from 17311 Mohs code; pathology is bundled into Mohs professional fee.	!HIGH 1 Biologic without PA Administering \$1,500+ biologic without prior auth on file; full denial + clawback risk.	!RED 1 Cosmetic billed to insurance Botox, fillers, peels, laser hair removal billed to insurance as medically necessary; insurance fraud exposure.
AMBER 2 Lesion count overstatement Billing 17003 (additional lesion) for lesions not actually destroyed or documented in note.	AMBER 2 Mohs on trunk/extremities without medical necessity Medicare covers Mohs on face/hands/feet/genitalia; trunk/extremities requires documentation of why standard excision inadequate.	MEDIUM 2 Step therapy non-compliance Biologic PA denied because prior methotrexate/cyclosporine/phototherapy trials not documented.	AMBER 2 Cosmetic + medical same encounter Billing both under same encounter; triggers fraud audit and potential Stark Law exposure.
GREEN 3 Benign vs premalignant miscoding Using 17000 (premalignant AK) for benign SK without biopsy confirmation.	LOW 3 Global period billing Billing E/M for related follow-up during 90-day Mohs global without modifier 25.	LOW 3 Site-of-service violations Billing biologic infusion as office-based when actually delivered in hospital infusion center (or vice versa).	LOW 3 Cosmetic drug code misuse Using J0585 (Botox) for chronic migraine (medical) without proper diagnosis code supporting.



OIG Work Plan 2025–2026 explicitly lists modifier 25 misuse in dermatology and cosmetic-medical billing separation as active audit targets. Medicare RACs have recovered **> \$120M** from derm practices since 2020.



■ CLOSING • KEY TAKEAWAYS

Three Actions to Capture Dermatology Revenue

For dermatology practice administrators and revenue cycle directors leading the next 90 days.

01



Automate biologic PA workflow

Build EHR template that auto-submits PA at biologic scheduling. Document step therapy (methotrexate/cyclosporine/phototherapy failures) before first PA submission. Pre-auth denial rate drops from 32% to <8% — recovering ~\$605K per 350-patient biologic panel.

PA DENIAL ~~32%~~ → < 8%

02



Enforce modifier 25 documentation

Audit last 200 E/M + procedure same-day claims for modifier 25 documentation sufficiency. Implement EHR template requiring separate E/M note when modifier 25 billed. Target: <5% denial rate (industry avg 16%).

DENIAL RATE ~~16%~~ → < 5%

03



Separate cosmetic from medical billing

Build separate cosmetic encounter templates in EHR. Require separate consent, separate ledger entry, separate payment processing for cosmetic procedures. Eliminates fraud exposure and recovers previously unbilled cash revenue (~\$400K/year per 1,000 cosmetic procedures).

RECOVERED CASH ~~\$0~~ → \$400K / yr



Questions & Discussion

Optimizing Dermatology Revenue Across E/M, Mohs, Biologics & Cosmetic

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